Fulvestrant · injections every 4 weeks

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

Hormone therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Injection area

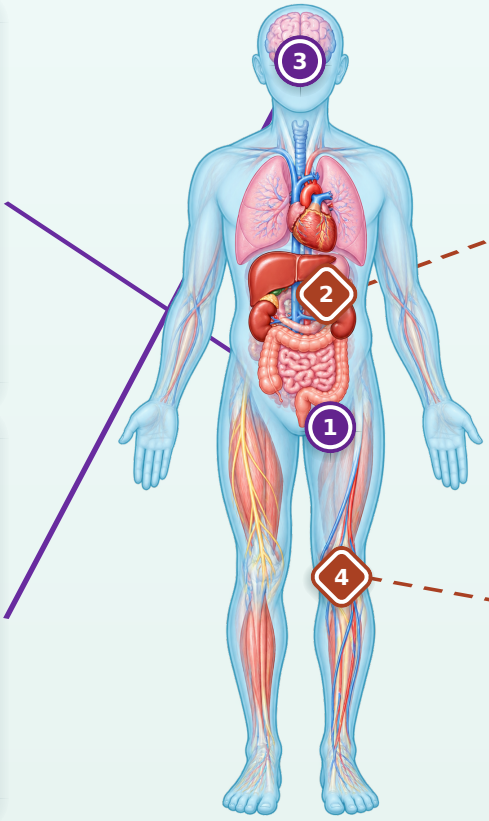
Fulvestrant

Report severe gluteal pain, numbness or pain travelling down the leg.

3 Hot flushes

Fulvestrant

Tell the team if flushes or sweats disturb sleep and daily life.



2 Liver

Fulvestrant

Report jaundice, dark urine or marked new illness; keep indicated liver tests.

4 Joints / muscles

Fulvestrant

Report persistent pain or symptoms limiting activity.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Tell the team about unusual bleeding or extensive bruising, injection-site pain with numbness or pain down the leg, jaundice or worsening symptoms. Severe chest pain, breathlessness or stroke symptoms require emergency help. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

YOUR PRACTICAL TREATMENT PLAN

Know the next step.

Taking treatment

NCCP schedule: 500 mg on Days 1 and 15 of the first 28-day cycle, then Day 1 of each 28-day cycle. This means Day 29 next, then every 28 days.

Each usual 500 mg dose is given as two 5 mL injections, one in each buttock, slowly over one to two minutes per injection. This is not an intravenous infusion; no port or pump is needed for fulvestrant itself.

Checks and follow-up

FBC, renal and liver profile. Confirm the indication, menopausal state, liver category, pregnancy exclusion and any bleeding / anticoagulant risk before intramuscular injections. Liver profile as clinically indicated. At injection visits review symptoms, bleeding / bruising and injection-site or nerve symptoms; response imaging follows the cancer-control plan.

The whole-treatment map distinguishes early-breast follow-up from advanced-disease response assessment.

Support between reviews

Symptoms and daily life

Report injection pain with numbness or radiating leg pain, extensive bruising, jaundice or worsening symptoms.

Support medicines

No routine antiemetic, steroid premedication or G-CSF is required by this endocrine monotherapy. Ask for individual symptom support.

Tests and appointments

Ask who books each check, when it is due and who reviews it. Tell the team if an expected appointment has not arrived.

Emergency: 112 / 999. Severe chest pain, major breathing difficulty, collapse or stroke symptoms need emergency help.

Contact your treating team promptly: Tell the team about unusual bleeding or extensive bruising, injection-site pain with numbness or pain down the leg, jaundice or worsening symptoms. Severe chest pain, breathlessness or stroke symptoms require emergency help.

Local · MON–FRI 8AM–5PM

Insert numbers here

Acute Oncology Service

Local · WEEKENDS / BANK HOLIDAYS 9AM–5PM

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your agreed out-of-hours pathway

Other hospitals: use the contacts your own team provides.

Your complete treatment reference

https://sactcheck.com/patient/00361/

Public information only. No personal information is encoded.